HOSPITAL DISCHARGE SUMMARY

Synthetic case: DEMO-PULM-3310 | Admission: 2026-04-10 | Discharge: 2026-04-13
Facility: Fictional North River Medical Center | Service: Hospital Medicine

REASON FOR ADMISSION

Fever, productive cough, pleuritic right-sided discomfort, and reduced oral intake.
Symptoms began approximately four days before admission, according to the case narrative.

DISCHARGE DIAGNOSES

1. Community-acquired pneumonia, right lower lobe - documented as the principal diagnosis.
2. Mild dehydration - improved after intravenous fluids.
3. Asthma history - no acute exacerbation documented during this admission.

RELEVANT HISTORY

The synthetic record lists asthma and seasonal allergic rhinitis. No chronic oxygen use reported.
Medication allergy: rash after trimethoprim-sulfamethoxazole, per patient history.
No penicillin allergy is listed in the supplied record.

HOSPITAL COURSE

Initial chest radiograph described a patchy right basilar opacity. The treating team documented that the presentation was consistent with pneumonia; viral infection remained in the differential.
Blood cultures showed no growth at 48 hours. Respiratory viral panel was negative for the targets tested. No organism was identified. Oxygen saturation remained above 94% on room air before discharge. The note states there was no hypotension or confusion.
Treatment included intravenous fluids and empiric antibiotics; see medication reconciliation.

HOSPITAL DISCHARGE SUMMARY - CONTINUED

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DISCHARGE MEDICATIONS

Amoxicillin 1,000 mg by mouth every 8 hours for 2 more days; complete the documented course.
Azithromycin 250 mg by mouth once daily for 2 more days; final dose listed for 2026-04-15.
Albuterol inhaler, 2 puffs by inhalation every 4-6 hours as needed for wheezing.
Acetaminophen 500 mg by mouth every 6 hours as needed for pain or fever; maximum per label.
No dose change to the patient's listed maintenance inhaled corticosteroid is documented.

STATUS AT DISCHARGE

Afebrile for 24 hours; eating and drinking; ambulatory without assistance. Cough persists but is reported as improved. These are documented discharge observations, not a guarantee of recovery.

FOLLOW-UP AND RETURN PRECAUTIONS

Primary-care follow-up requested within 5-7 days. Repeat chest imaging in approximately 6 weeks may be considered if symptoms persist, per the discharge note. This is a recommendation, not an appointment already booked. Seek urgent assessment for worsening shortness of breath, blue lips, new confusion, inability to keep fluids down, or recurrent high fever, as written in the note.

END OF REPORT - Synthetic educational test fixture; not a real patient record.